achieving for children

Transfer Review Report – Psychological Contribution

The following advice is provided to the Local Authority to assist them in the transfer of Drew Hick's Statement of SEN to an Education, Health and Care Plan (EHCP), following Educational Psychology involvement during two visits to Canbury School and phone consultations with Mrs Hicks and school. The psychological advice is provided in accordance with the Children and Families 2014 Act and the SEND Code of Practice: 0 – 25 years (2014).

1. Personal Information	
Child/young person's name: Drew Hicks	
Date of birth: 08.08.2004	**Age: 12 years, 9 months (12:09)**
Home address: 16 Winchendon Road, Teddington, Middlesex. TW11 OSX	
Parents/carers: Mr and Mrs Hicks	
Home language: English	
School: Canbury School	**School Year Group: 8**

2. Purpose of report:

This report has been requested by Richmond Local Authority to support them in their assessment of the special educational needs and disabilities (SEND) of Drew Hicks under the 1996 Education Act and in according with the Children and Families Act 2014.
It is therefore understood that the intended audience of this report are:

- parents/carers of Drew Hicks;
- those responsible for co-ordinating the assessment and drafting of an Education, Health and Care Plan (EHCP);
- other professionals contributing to the assessment and plan.

3. Reason and aim of Educational Psychology (EP) involvement (desired outcome):

Educational Psychology involvement was requested by the London Borough of Richmond upon Thames to provide an updated assessment of Drew's needs to assist with the transfer of her Statement of Special Educational Needs to an Education, Health and Care Plan and to support the identification of appropriate educational provision.

This report outlines the key findings following my involvement; it identifies Drew's strengths and explores information that helps to explain the barriers and challenges to learning that she is experiencing. It is important to note that whilst this psychological report represents a picture of Drew's needs, it is based on information gathered within particular contexts. It should be read in conjunction with other sources that provide information about Drew's development and further evidence of how she manages within different settings and across a range of contexts.

In order to understand Drew's needs, the reader's attention is drawn to the extensive evidence presented in the accompanying professional reports and information provided by home and school that has been submitted as part of the transfer to an Education, Health and Care Plan regarding Drew's complex developmental, mental health and emotional well-being needs. I would recommend that this information is read in its entirety, in conjunction with the current report.

4. Timeline of EP involvement and sources of information:

Date:	***Nature of Involvement:***
May 2017	Reading of professional reports.
31.03.2017 and 21.04.2017	Telephone conversations with Mrs Hicks (Mother).
08.05.2017	School visit including a consultation with Mrs Hicks and Ms Branney (Academic Assistant Head and Geography Teacher) class observation, and one-to-one session with Drew.
08.05.2017	Discussion with the Single Point of Access Team (SPA).
08.05.2017	Telephone conversation with Mrs Hughes.
08.05.2017	Telephone conversation with Ms Branney.
10.05.2017	School visit including a follow-up consultation with Mrs Hicks and Ms Branney and assessment with Drew.

5. Relevant background information:

An in-depth description of Drew's life to date is documented within the accompanying professional documentation and from parental reports. The full contents of this will not be repeated here, however some pertinent events will be outlined. For a full summary, please contact Mr and Mrs Hicks.

Drew attends Canbury School, a small independent secondary school in Kingston. Drew is currently in Year 8. Drew lives with her adoptive parents, and was adopted as a baby when she was five months old. Drew reportedly knows that she is adopted. Drew did not know her biological parents, however has had previous direct contact with her father, one of her half siblings and her grandparents. Drew was reportedly 'born at 38 weeks, having been exposed in utero to multiple Class A drugs'. As described in Dr Lane's (Child and Adolescent Psychotherapist) report (dated 19th May, 2016) 'Drew was born with Neonatal Abstinence Syndrome, and was in intensive care for 10 weeks'. Drew's biological mother is reported to have died three years ago, and Drew attended the funeral.

During the onset of puberty, Drew experienced challenges understanding her identity. For a time, Drew identified as transgender, and referred to herself as male and with male pronouns. Mrs Hicks reports that this has since abated, and Drew appears significantly more comfortable with her female identity. Drew is known to The Tavistock and Portman Gender Identity Development Service (GIDS) and appointments were offered, however the family feel that Drew's gender identity does not currently present as a need.

Mrs Hicks explained that in March 2016, Drew took an overdose of paracetamol, which resulted in an admission to hospital. Drew has reportedly engaged in self-harming on four different occasions, however this has reportedly not occurred for over a year.

Drew was referred to the Neurodevelopmental Team at Springfield Hospital in May 2016 for an assessment, conducted by Dr Theivendran, Specialist Registrar in Child and Adolescent Psychiatry, and Syed Miah, Assistant Psychologist. The assessment concluded diagnoses of both Attention Deficit Hyperactivity Disorder (ADHD) and Autism Spectrum Disorder (ASD). Dr Theivendran reports that, "The nature of some of the symptoms of ADHD will mask some of the ASD features. Drew appears to be socially interested and engages in friendships which is typical of girls with ASD. There may be some degree of attachment difficulty also complicating the picture". This highlights the complexity of Drew's presentation of needs, and she requires intervention and support to be carefully tailored and personalised in order to serve a

supportive function and meet her needs. Drew currently takes Melatonin daily to aid sleeping, and is trialling a third medication to manage her ADHD, which is being monitored regularly by CAMHS.

The family engage with a number of services, including: ongoing appointments with Dr Doig, Consultant Child and Adolescent Psychiatrist at the Child and Adolescent Mental Health Service, and Janice Oakley, Independent Speech and Language Therapist, attached to the school. In addition, Drew had an Occupational Therapy assessment conducted by Emtia Bartlett, Children's Occupational Therapist, on 04.04.2016. Furthermore, Drew and Mrs Hicks attended attachment focused therapy between May 2016 and March 2017 with Louis Sydney, Child and Adolescent Psychotherapist and Adoption Consultant. Furthermore, the family have recently been referred to the Family Support Team with Mrs Hicks' consent, to explore further support available.

6. Child/young person's views/aspirations *(in addition to those documented elsewhere in the report)***:**

Drew presented as amenable and polite, and appeared happy to engage in conversation with me. When asked what was going well in her life, Drew explained that she likes English, because her class teacher 'jokes and has banter' and explains clearly what to do. This approach appears to work well with Drew and was witnessed during my observation of a Maths lesson; she appeared more able to concentrate and focus on the task at hand, when she had received clear direction and encouragement. Drew also explained that she likes writing stories, Art, DT, PE and Science and explained that 'friendships are going well'. However, Drew went on to explain that she can get annoyed with her peers and would 'punch that person' if they annoyed her. She explained that she can get angry and 'would like to change schools'. Drew explained that it can be a long day at school, and that homework is 'tricky'.

When asked to scale how she was feeling (1=the worst things could be and 10=the best things could be) she explained that she currently sits at a 4.5. When asked to elaborate why she feels at a 4.5 rather than a 3.5, she explained that it is 'not the worst'. When asked how others would describe her, Drew used negative adjectives to describe her character, perhaps suggesting that she has a lack of self-worth and self-esteem.

Drew described her dog, whom she 'loves' and has a very close relationship with. She also said that she enjoys playing the guitar and her ukulele. When asked what helps her when she finds things tricky, she explained that her 'support dog', chart, and 'spinny object' helps. Drew's aspirations reportedly include going to Art College in California.

Drew presented as a bright, amenable and polite young person. *It will be important for those working with Drew, to actively involve her in target setting and to gain her views regarding her provision of support, in order to help to foster self-confidence and equip her with some autonomy in decision-making.*

7. Parent/carer's views/aspirations *(in addition to those documented elsewhere in the report)***:**

Mr and Mrs Hicks have a clear view regarding Drew's developmental and emotional needs, and wish predominantly for her to become happier and able to live a healthy and content life, fulfil her potential, and have a happy and inclusive future.

Mrs Hicks expressed great concerns regarding the notable change in Drew's mood during Secondary school, namely Drew's complex emotional needs, including previous suicidal ideation, and her difficulties accessing, and engaging with, school life. Home life has also become extremely difficult, with Drew demonstrating emotional fragility and significant volatility and changes in mood. This is

understandably extremely difficult for Drew's parents, and they strongly wish for her to be supported to be able to cope with the demands of life and to reach her potential.

Mrs Hicks explained that Drew requires appropriate support in order to maximise her potential and enjoyment of school, and allow her to succeed and utilise her strengths. Mrs Hicks explained that a routine which allows for flexibility, suits Drew's needs best. Furthermore, Mrs Hicks described her hopes for future provision, including explaining that Drew needs 'a consistent partner in the form of an LSA to support her, and to provide continuity in order to maximise her learning to become independent, and be able to self-manage'.

Mrs Hicks appears very receptive to ongoing support from relevant services, and is actively exploring parenting programmes and groups run by the National Autistic Society in an effort to learn helpful strategies, to develop her knowledge of Autism further, and as a way of connecting with the parenting community.

8. Learning and development – strengths and barriers:

This section outlines Drew's strengths and needs related to her ability to access learning, the strategies that are currently effective in facilitating this learning, and what still needs to be achieved in order to allow Drew to access a rich and varied curriculum.

Cognition and Learning

Drew has been described as capable, and she experiences particular success in Art and Science. Drew responds well to chunking of information, short steps, repetition and reinforcement of information to support her learning. Drew is described as being 'aware of her limitations' and has reportedly requested additional support, demonstrating maturity and self-concept as a learner. Drew possesses relative strengths in her comprehension skills.

Drew can demonstrate impulsivity in the classroom by shouting out, and acting without carefully thinking through her actions. Furthermore, she has been described as displaying 'immature' behaviour at times, and she can display variable behaviour in the classroom, which may serve the function of receiving additional support and attention from others perhaps. Drew reportedly experiences significant literacy difficulties, and this has an impact on her ability to engage and progress with English. Drew is described as a 'reluctant reader', and experiences notable challenges with her spelling. 'Drew's writing reportedly does not match her verbal contributions', as noted in the annual review document. Drew has one-to-one sessions with a Specialist Literacy Teacher for one hour per week to support her literacy acquisition. Furthermore, Drew reportedly has 'weak executive functioning and working memory' and has a reduced capacity to organise her work and belongings both at home and at school. Drew reportedly does not yet know her times tables, and possesses difficulties with her attention and concentration, requiring stepped activities and adult direction to help to keep her on task.

Drew can find the demands of classroom learning a challenge, particularly in subjects such as Physics, where she perceives there to be a difficult relationship with the teacher which can prevent her from engaging and attending to the work. In addition, Drew finds homework a source of anxiety and this, in turn, can become a challenge for the family to manage. Drew attends homework club and receives support and encouragement, however she reportedly requires support to promote her confidence, motivation and self-esteem, so that she develops her self-efficacy and belief that she can succeed.

In order to explore Drew's approach to learning further, I asked her to complete the WASI II[1] with me (a full description of the assessment is included in the Appendices). This is a shortened version of the WISC-IV UK, which I felt would be more suitable for Drew as her attention difficulties would be less likely to affect her ability to demonstrate her capability. This was carried out in a one-to-one situation after Drew had already met me, to ensure that she was as relaxed as possible. The results should therefore show Drew's learning capabilities to the best of her ability. The discrepancy between these scores and her achievements at school are likely to largely reflect the extent to which Drew's difficulties discussed above are impacting on her learning in the classroom.

- The assessment indicated that Drew has **'superior' (above high average) verbal ability (94th percentile[2]).** Drew demonstrated the ability to easily follow instructions, and process and reason with words. Drew appeared calm and reflective when talking one-to-one with an adult and is articulate when describing her thoughts.
- Drew has **high average non-verbal ability (79th percentile).** She has superior ability to manipulate physical objects in order to match a visual display (Block Design subtest) demonstrating an ability to easily find and copy patterns in pictures. However, she achieved a score in the average range for the 'Matrix Reasoning' subtest, whereby she had to look at an incomplete matrix and select the missing portion from five response options. Although Drew's score still fell within the average range, there was a significant discrepancy between this score and the Block Design score, perhaps indicting fatigue at this point in the assessment, or that she finds perceiving visual information more of a challenge. It was observed that she appeared to answer rather impulsively and quickly at times as the subtest became increasingly more complex, perhaps indicating that she was not paying close attention to the rules of the subtest.
- The assessment indicated that Drew has **'superior' overall cognitive ability (92nd percentile**) indicating significant strengths within her cognitive profile.
- **Drew's school achievements do not currently match her cognitive ability.** It was explained that currently Drew is below average in Maths, markedly below average in English, however is achieving within average for Science. Given Drew's cognitive ability as suggested by this assessment, Drew should be achieving in line with her peers and commensurate with her ability in all subjects.
- Drew's difficulties with attention, concentration, literacy, motivation and self-esteem are all, cumulatively, having an impact on her academic achievement.
- Drew appeared engaged and motivated throughout the assessment. Drew exclaimed at the end that 'it was easy and hard...as it progressed it got harder'. Mrs Hicks explained that the one-to-one situation with minimal environmental distractions, coupled with the medication that Drew is taking in school to manage her symptoms of ADHD, makes a significant difference to her ability to attend to, engage with, and concentrate and focus on learning tasks.

Communication and Interaction

Drew's verbal communication skills have been described as a strength, and she is reported to 'be on a par with her peers'. Drew is able to engage in conversations with adults and peers, however she may benefit from ongoing support to facilitate her social interactions and development of age-appropriate language when engaging in conversations. Janice Oakley, Independent Speech and Language Therapist, reports that 'Drew listens and follows the rules well, and is happy to share conversations on a range of topics'. The report also outlines that 'the content of Drew's conversation can be a little extreme',

[1] Wechsler Abbreviated Scale of Intelligence II: a short cognitive assessment for verbal and non-verbal ability.

[2] The percentile scores reveal the percentage of young people who would be expected to achieve a higher or lower score. For example, where Drew achieved a score at the 94th percentile for Verbal Comprehension, this suggests that 94% of young people (or 94 out of 100) of the same age would achieve a lower score, whilst 6% would achieve higher. Percentiles range from 1 – 100, with the 50th percentile as the average. Scores between the 25th and 75th are considered within the average range.

indicating that Drew is not always able to adapt her language accordingly and shows some vulnerability and variability in her communication. Drew receives one half an hour session per week of group Speech and Language Therapy.

Drew was formally assessed using the CELF (Clinical Evaluation of Language Fundamentals) by the Speech and Language Therapist in November 2015, and her scores were found to 'fall generally in the low average range, with no specific area of difficulty'. Therapy has focused upon the development of Drew's listening and attention skills, comprehension of more complex language, precise use of vocabulary and grammar to express meaning and the development of mature conversation skills. During our assessment, when asked to describe the word 'impulse', Drew explained that it means 'jumping off a cliff as an adrenaline rush, like on Twilight'. Upon further exploration, Drew explained that she would not want to do this as 'it would hurt', but that she has seen it on TV. This pertains to Drew's significant vulnerability and propensity to engage in risky behaviours, whereby she may not understand the consequences or her actions. This also indicates that Drew's understanding of language can be literal in nature, therefore suggesting that clear and concrete language should be adopted by those supporting her. Drew requires a high level of support to scaffold her conversation, consider scenarios and appropriate responses, and mentoring on safe behaviours. Drew reportedly 'does not respond to firm voices', therefore this has implications for those supporting her at school. A supportive, personable and encouraging approach is likely to best engage Drew.

Social, Emotional and Mental Health

Drew reportedly requires significant support, supervision and 'therapeutic parenting' to navigate daily life. Louis Sydney, Child and Adolescent Psychotherapist and Adoption Consultant, reported that Drew 'requires support (both professionally and from her parents) in terms of her cognitive, emotional, psychological, physiological and social development on a day to day basis'. Drew experiences significant emotional fragility and vulnerability, and has had periods of engaging in self-harming, including taking an overdose on one occasion in March 2016. This risky behaviour has abated over the last year, and Mrs Hicks described 'a happier girl more visible at home', however emotional outbursts still occur more frequently at home than at school, and Drew can become 'hyper-aroused' when upset. Drew responds well to being given space at such times. Drew has also started attending group mindfulness sessions at school each week to assist her emotional regulation, which she is reportedly enjoying. Drew attended a total of thirty-three sessions with Louis Sydney, however reportedly found it 'difficult to engage in the therapeutic process'. Mrs Hicks explained that a decision was made to end the therapy, as Drew began to disengage from the process and expressed that she wanted to stop the sessions.

Mrs Hicks explained that Drew does not offer physical affection or emotional warmth towards her parents. Mrs Hicks explained that she and Mr Hicks have adapted their parenting style to support Drew's emotional regulation needs. Furthermore, Drew can lack empathy with others, and often does not appear to understand others' feelings or emotions. Drew is reported to enjoy break-time and interacts with her peers. However, it was described that Drew 'flits' in her peer group, and can use inappropriate language for her age during interactions with others.

Drew's thinking patterns can be rigid and entrenched, which are synonymous with her diagnosis of ASD. This therefore indicates that it may be challenging to break the cycle of anxiety that Drew may be experiencing, as it may be self-perpetuating for her. Drew may experience a sense of 'learned helplessness', whereby she feels helpless to avoid negative situations because she may feel that she has no control over the anxiety provoking situations. Drew was witnessed to say 'now what do I do' a number of times in the lesson which I observed, and she often looked at her peers for guidance in class.

This may suggest a lack of agency and self-motivation over her learning, and Drew requires targeted support to increase her self-concept and confidence as a learner.

Mrs Hicks explained that Drew can experience anxiety-related stomach pains. School and home indicate that progress in her subjects has been adversely affected by her complex needs, and this can result in disengagement with learning, causing her to be unable to fulfil her learning potential. School and home explained that Drew requires ongoing emotional support, pre-warnings about change, a sensory reduced environment, social skills support and a nurturing environment which empathises with her complex needs and provides the support, motivation and encouragement that she needs in order to be able to fulfil her potential.

Physical and Sensory

Drew experiences a significantly disrupted sleep routine, and takes Melatonin on a daily basis to assist with her sleep hygiene. Drew's sleep routine can subsequently significantly impact upon Drew's parents, and this makes functioning well each day a challenge for the family. Drew is currently undergoing a third trial of medication under the care of CAMHS, and this can suppress her appetite. Drew reportedly experiences some 'tics', which are thought to be aggravated by her medication. Furthermore, Drew is described as needing frequent prompting to brush her teeth as part of her self-care routine at home.

Drew is reported to possess particular sensory sensitivities, including disliking eating at school. Drew can 'head bang', which was evident during my observation when Drew emulated this movement onto her desk. She also explained to me that she does this as it feels good, however knows that her parents would warn her from doing it. Emtia Bartlett, Children's Occupational Therapist, explains in her report: 'It is probable that she is still under-responsive to vestibular processing'. It was noted during observation that she did have a tendency of slouching and rocking on her chair. Emtia Bartlett explains that 'Drew has made progress with her gross motor skills', however states that Drew 'should continue with participating in a wide range of gross motor activities, as these assist in developing balance and core strength'. No particular current concerns were reported by home or school in this area.

Drew can find it challenging to record her ideas in writing, and this can become a source of anxiety for her. Emtia Bartlett notes that Drew has 'hypermobility in her fingers, and this does impact pencil control'. Drew receives weekly fine motor skills sessions in groups with her peers to support her fine motor and handwriting development.

9. Conclusions

Drew is an amenable, charismatic and articulate twelve-year-old, who has great potential. She has diagnoses of Attention Deficit Hyperactivity Disorder (ADHD) and Autism Spectrum Disorder (ASD) which can cause her to experience difficulties with social interaction and communication. Drew experienced traumatic early life experiences and was adopted as a baby. Drew possesses complex emotional needs, including experience of low mood, suicidal ideation, anxiety, attachment difficulties and previous gender dysphoria. Of particular concern are Drew's anxieties, vulnerability and propensity to engage in risky behaviours. This has had a detrimental impact on family life, and Drew's mother is extremely worried about the family functioning and Drew's future well-being and happiness.

Drew's needs mean that she is falling behind her peers academically, and is unable to currently fulfil her considerable potential. The cognitive assessment conducted with Drew indicates that she has significant strengths in her verbal comprehension and non-verbal perceptual reasoning skills, achieving scores in the above average range. Drew has experienced significant adjustments in her life in relation to her

early life experiences and attachment, previous gender identity exploration, and recent diagnoses of ADHD and ASD.

Drew has low confidence and self-esteem, particularly in relation to her ability to demonstrate her strengths in learning. She can struggle to cope with the unpredictability and changing expectations of different lessons and teachers in mainstream education. Changes, transitions, and sensory over-stimulation, can create particular discomfort for Drew. Drew experiences disrupted sleep, for which she takes medication, and has a restricted diet. The small setting of Canbury School is reported by Mrs Hicks as highly conducive in meeting Drew's particular needs. Drew appears to find comfort in spending time with her dog and her family, however the quality of these times have been affected over time due to the decline in Drew's emotional well-being, affect and overall functioning.

Drew will benefit from considerable emotional and practical support and encouragement, in order to attend to and engage with learning, and to manage the demands of school life and social interactions. It will likely be helpful to aim to use a consistent approach, key motivators, and activities which Drew particularly enjoys, in order to aim to engage her interest in other curriculum areas. Drew will require teachers and support staff who understand her complex range of needs, sensitively support the development of her identity, and who are able to differentiate, scaffold, and adapt her work and environment appropriately and sensitively, to enable Drew to flourish and achieve her potential.

10. Key strengths and educational needs, outcomes and recommendations:

The following outcomes are based on the Educational Psychologist's perspective from individual work with Drew and consultation with Mrs Hicks and Ms Branney at Canbury School. Due to the timescales involved in this piece of work, there has not been opportunity for collaborative discussion regarding outcomes across all involved parties. School have suggested a number of outcomes, which have been incorporated within this report. It is recognised that the outcomes identified are broad in nature, in order to allow for a full description of achievable outcomes with the other members of the network. As a result, further consultation with Drew and relevant professionals will be required to ascertain and refine agreed outcomes based on Drew's specific needs and aspirations.

Category of strengths	Detailed description of strengths
Cognition and Learning	▪ Drew can present as a bright, amenable and polite young person. ▪ Drew has notable strengths in verbal comprehension and perceptual reasoning, as indicated in the cognitive assessment. ▪ Drew has a passion for painting and Art. ▪ Drew has been described as capable, and she experiences particular success in Art and Science. ▪ Drew responds well to chunking of information, short steps, repetition and reinforcement of information to support her learning. ▪ Drew is described as being 'aware of her limitations'. ▪ Drew possesses relative strengths in her comprehension skills.
Communication and interaction	▪ Drew's verbal communication skills have been described as a strength, and she is reported to 'be on a par with her peers'. ▪ Drew is able to engage in conversations with adults and peers. ▪ Drew was able to engage in reciprocal conversation during assessment. ▪ Drew was observed to interact with her peers during break-time.
Social, emotional and mental health	▪ Drew responds well to being given space and time to regulate and reflect. ▪ Drew has started attending group mindfulness sessions at school each week to assist her emotional regulation, which she is reportedly enjoying. ▪ Drew is reported to enjoy break-times and interacting with her peers. ▪ Drew displayed a sense of humour during our one-to-one sessions.

Physical and/or Sensory	▪ Drew has made progress with her gross motor skills and shows relative strengths in this area.

Category of need *(SEND Code of Practice)*	**Detailed description of need**	**Outcome to be achieved**	**Recommendations for achieving this**
Cognition and Learning	▪ Attention and concentration difficulties, and a propensity to act with impulse. Drew has a diagnosis of Attention Deficit Hyperactivity Disorder, characterised by impulsivity and a reduced ability to sustain concentration and focus. ▪ Difficulties with engagement, attendance and preparedness for learning. ▪ Drew's school achievements do not match her cognitive ability. ▪ Drew experiences difficulty in literacy acquisition and her progress in reading, writing and spelling is slow**.**	▪ By May 2018, Drew will be able to make progress in learning, at least in her core subjects, over the course of the next year. Drew will be able to fully engage in lesson content and activities, with adult support. ▪ By May 2018, Drew will become more independent with her approach to homework, and will attend and participate in after school homework sessions. ▪ By May 2018, Drew will be able to complete a self-directed piece of work independently with clear set targets, once per day. ▪ By May 2018, Drew will be able to sustain focused attention in lessons for at least the first five minutes on a daily basis.	▪ Provision whereby adults are skilled and experienced in working with young people with autistic spectrum conditions and social communication difficulties will be beneficial. ▪ Key information about Drew's complex emotional needs and vulnerabilities should be circulated amongst all staff involved in Drew's support in the education setting. It will be important to share a 'learning passport' to help aid smooth and successful transitions, and reduce the level of anxiety surrounding change and transitions. Drew should be closely supported socially, emotionally and academically, in order to support her engagement in education. ▪ Drew requires adult support to focus her at the start and end of each lesson. Strategies to encourage Drew to maintain focus on a task will enable her to increase her level of concentration on learning tasks. Drew may benefit from regular verbal reminders and prompts to focus, and all tasks should be set at an appropriate level with suitable materials available. ▪ Drew should be, as much as possible, involved in planning her individual learning programme and take some agency over this in order to aim to increase her

			motivation and engagement. She is an intelligent and reflective student, who is capable of contributing to setting goals and targets. ▪ It will be important for Drew to be given learning opportunities in which she can experience success, in order to raise her self-esteem, confidence and motivation to attend. ▪ Drew requires learning activities to be broken down into small steps and information to be reinforced and repeated. ▪ Drew requires a Key Worker at school with whom she can build a strong positive working relationship. A consistent, empathetic and supportive approach is of significant importance. This adult should support her to consider her thoughts, challenges, and apply strategies for overcoming these. ▪ Drew requires strategies to be explicitly taught to her, as well as when to apply them and how to monitor which strategies are most effective for her. It is important for any strategy taught outside of the classroom to be reinforced within the classroom, by effective communication between support staff and teaching staff. ▪ Drew requires learning to be appropriately differentiated to support her needs and encourage participation, whilst being appropriately challenging. ▪ Drew requires ongoing support in the classroom as well as in small groups to continue to develop her literacy skills. ▪ Drew will require the opportunity to utilise computer-based and electronic resources as well as creative and physical materials during activities in order to consolidate knowledge and encourage engagement with learning (i.e. multi-sensory approaches to learning).

Communication and interaction	▪ Drew has a diagnosis of Autism Spectrum Disorder, characterised by rigidity in thinking, communication difficulties, and difficulties expressing feelings and empathising with others. ▪ Developing age-appropriate language when interacting and communicating with others. ▪ Receptive language difficulties, involving a literal understanding of language.	**Further specific outcomes relating to the development of Drew's language and communication skills may be advised by the Speech and Language Therapist; however, the following may be considered:** ▪ By May 2018, Drew will meet with a trusted adult in school on a daily basis to review and monitor progress, and reflect upon aspects that have gone well and areas for improvement each day. ▪ By May 2018, Drew will be able to engage in a small group learning activity in an area of shared interest with a minimum of one other carefully selected peer (with guidance from an adult) where she can be encouraged to draw upon her ability to contribute her ideas. ▪ By May 2018, Drew will be able to discuss her thoughts with a trusted adult at school in a one-to-one situation.	▪ A structured programme of support may be required to help to continue to develop Drew's language and communication skills as advised by a Speech and Language Therapist. ▪ Drew will require opportunities to engage in conversations that align with her interests and things that she enjoys doing to support the development of her self-esteem and self-concept in relation to her social interaction skills. ▪ Adults at school may need to orchestrate structured fun activities with a couple of peers in a safe and non-threatening environment, in order to facilitate the development of Drew's social skills. ▪ Drew requires adults to be aware of their tone of voice, as Drew can be sensitive to this. Drew can also perceive language in a literal way, therefore adults should use clear, concrete and accessible language with Drew to avoid misunderstandings and misinterpretation.
Social, emotional and mental health	▪ Emotional regulation difficulties. ▪ Emotional vulnerability, attachment needs and	▪ By May 2018, Drew will be able to self-manage her emotions and utilise suitable known strategies to calm and self-regulate her behaviour.	▪ In order to ensure that Drew receives the most appropriate support in managing her needs, it will be helpful to ensure the continuing involvement of other agencies when developing her support plan, and the continuation of a coordinated approach to support her emotional needs.

	propensity to engage in risk-taking behaviours.	▪ By May 2018, Drew will be able to balance her thoughts around anxiety provoking situations with adult support, in order to learn, make meaningful friendships, and participate at school. ▪ By May 2018, Drew will be able to explain how she is feeling using words, visual images or drawings on at least one occasion per day. ▪ With the support of a learning mentor, Drew will continue to develop a positive self-image and self-esteem, and will identify successes in school and be able to attribute successes to her own positive actions, with guidance from an adult. ▪ Drew will experience an improvement in her overall emotional well-being and mental health. Drew will feel safe at school, and will enjoy time with her peers.	▪ A sensitive approach will need to be adopted by all those working with Drew, in order to support her with developing her sense of identity. ▪ A risk assessment of the school environment should be undertaken to ensure Drew's safety and awareness of danger and risk. ▪ A vigilant approach should be adopted by all those supporting Drew, regarding potential risk-taking behaviours. Drew is a vulnerable young person and all adults involved in supporting her will need to ensure that they are vigilant of her safety and of situations in which she may be particularly vulnerable. ▪ Drew will benefit from sessions with a learning mentor, to support her in managing her emotions and to encourage her to reflect upon the difficulties she has faced and how she may overcome these. ▪ Drew will require support to develop a greater understanding of emotional cues, to enable her to manage her emotions and broaden her understanding of her own and others' emotions, and to develop some awareness of the physiological signs associated with her feelings. The use of visual resources are likely to be helpful to encourage Drew's emotional literacy skills. ▪ It is important that all adults understand the nature of Drew's difficulties and how this can impact on her, and implement appropriate strategies to manage this. Drew needs opportunities for relaxation in order to support her, particularly during times when she is upset. ▪ All those supporting Drew need to be aware of her vulnerability. She is likely to benefit from learning about strategies that can help her in social situations where potential difficulties could arise (including situations that might give rise to bullying).

			▪ Drew requires endings in relationships to be very carefully managed. She should be spoken to in a one-to-one situation with a trusted adult to prepare her and explain what is happening and why. Drew could be given something tangible, such as a card, when people leave to make her feel anchored, supported, and 'held in mind'. ▪ Drew may benefit from ongoing therapeutic support from the Child and Adolescent Mental Health Service, to further explore and support her emotional well-being and mental health needs, as deemed appropriate by that service.
Physical and/or Sensory	▪ Disrupted sleep routine. ▪ Sensory sensitivities, including a restricted diet and sensitivity regarding the food that she eats and where. ▪ Fine motor difficulties.	▪ By May 2018, Drew's sleep hygiene will be improved to align with the routine consistent with a school day, in order to feel prepared and well rested. ▪ By May 2018, Drew will begin to broaden her diet, and eat a healthier and more balanced diet, on a daily basis. ▪ By May 2018, Drew will be encouraged to record her ideas in writing with increased clarity, on a daily basis.	▪ It may be helpful for home to request the support of the GP to explore Drew's dietary and self-care needs further. ▪ Drew will benefit from a known quiet, calm space to go to whenever she feels particularly overwhelmed by her sensory environment, and to access during break-times and lunch-times. ▪ Drew will require a small and calm environment that is conducive to her sensory needs in order to reduce sensory overload and anxiety. ▪ Alternative methods of recording could be implemented in school for Drew, at the discretion of those supporting her (e.g. use of a computer). Drew will also require targeted support to develop her ability to write with a pen/pencil.

This report has been produced by: Dr Sophie Casson
Educational Psychologist
HCPC registered

Signed: SACasson

Date of report: 18.05.2017

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cc. Mr and Mrs Hicks, parents; Gayle Branney, Canbury School; SEN; EPS File.

Appendix A - Wechsler Abbreviated Scale of Intelligence (WASI)

Extremely Low	Borderline	Low av.	Average	High av.	Superior	Very Superior
29 or less	30 – 36	37 - 42	43 - 56 ***t-score***	57 - 62	63-69	70 or more
2nd percentile	3rd – 8th	9th – 25th	50th **percentile**	75th - 90th	91st – 97th	98th percentile
69 or less	70-79	80-89	90-109 **standard score**	110-119	120-129	130 or more

Subtests (in the order of presentation)	Description	*t*-score
Block Design	While viewing a constructed model or a picture in the Stimulus book, the child uses red-white blocks to re-create the design within a specified timed limit.	65
Vocabulary	For Picture items, the child names pictures that are displayed in the Stimulus Book. For Verbal items, the child defines the words that the examiner reads aloud.	67
Matrix Reasoning	The child looks at an incomplete matrix and selects the missing portion from five response options.	50
Similarities	The child is presented with two words that represent common objects or concepts and describes how they are similar.	62

A total of three composite scores can be obtained with the WASI:

Composite	Sum of *t*-scores	Composite Standard Score	Percentile Rank
Verbal Comprehension Index	129	123	94
Perceptual Reasoning Index	115	112	79
Full Scale	244	121	92

Verbal Comprehension Index – composed of subtests measuring abilities that use reasoning, comprehension and concept formation. It consists of the sum of *t*-scores of the subtests Similarities and Vocabulary.

Perceptual Reasoning Index – composed of subtests measuring perceptual reasoning and organization. It consists of the sum of *t*-scores of the subtests Block Design and Matrix Reasoning.

Full Scale – composed of all four subtests, this gives an overall measurement of cognitive ability.